

Patient ID: NancyPeriod: April 25 to July 24, 2024Report Date: July 25, 2024Coverage: 1315/2184h (60%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

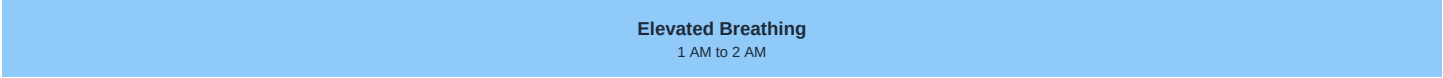
Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpmElevated HR > 95 bpmHigh HR > 100 bpm

Last 24 Hours



Vital Stat	Avg	Min	Max
Heart Rate	69 bpm	49 bpm	90 bpm
Breathing	17 brpm	10 brpm	39 brpm

Limited data in the final 24h (14/24h); values reflect available readings.

Last 24 hours: one elevated breathing episode overnight, otherwise within normal range.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
1 AM to 2 AM	1h	1	Elevated Breathing	24	18	39	Check SpO2 and lung sounds; assess for fluid overload or early infection

Patient clinical status over 91 days from Apr 25 to Jul 24

HR Breathing

Elevated Breathing	Elevated Breathing	Elevated Breathing	Elevated Breathing	Elevated Breathing	Elevated Breathing	Elevated Breathing	Low HR	Low HR
Apr 25 to Apr 26 (2d)	Apr 30 to Apr 30 (1d)	May 17 to May 17 (1d)	May 20 to May 20 (1d)	Jun 23 to Jun 24 (2d)	Jul 04 to Jul 04 (1d)	Jul 24 to Jul 24 (1d)	Jun 06 to Jun 07 (2d)	Jul 21 to Jul 22 (2d)

19 episodic events Detected, spanning 19h total. Conditions: Elevated Breathing, Low Heart Rate. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
19	19h	Low Heart Rate	<1	60%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jun 06	9 PM to 10 PM	2h	2	Low Heart Rate	45	42	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jul 21	3 AM to 4 AM	7h	7	Low Heart Rate	44	40	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 25	9 PM to 10 PM	3h	3	Elevated Breathing	25	23	26	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 30	9 PM to 10 PM	1h	1	Elevated Breathing	25	22	27	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 17	4 PM to 5 PM	1h	1	Elevated Breathing	27	27	27	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 20	12 PM to 1 PM	1h	1	Elevated Breathing	24	24	25	Check SpO2 and lung sounds; assess for fluid overload or early infection

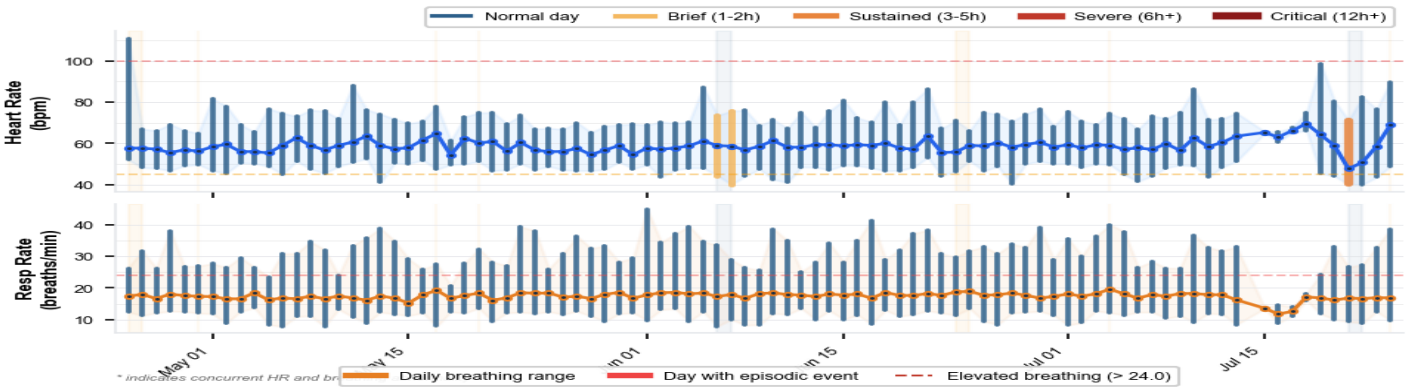
+ 3 additional condition(s) across Elevated Breathing; details in monitoring data.

Suggested Clinical Review Actions

- Low Heart Rate (avg 44 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 25 brpm, peak 26 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 9 unstable phases with 18 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

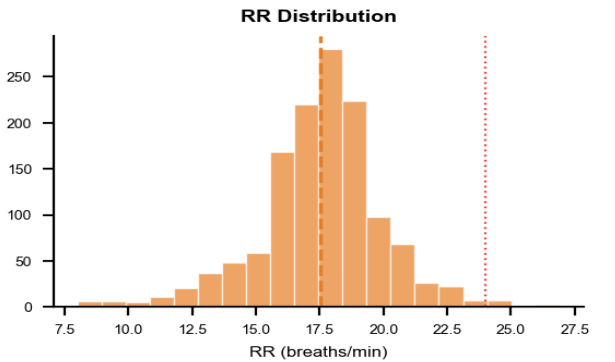
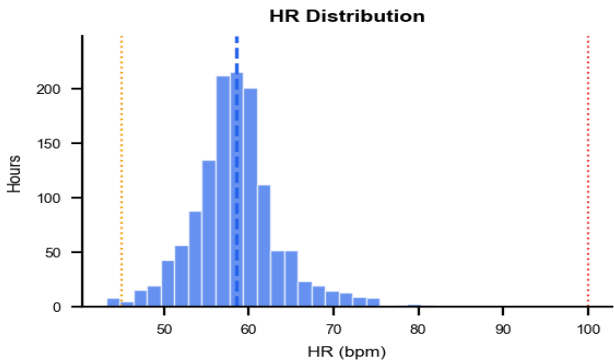
Clinical Pattern Observations:

- 1. **Clustered pattern:** Episodic events on 3 of 91 days (3%).
- 2. **Nocturnal pattern:** 4 of 6 eligible episodes during evening or night hours.

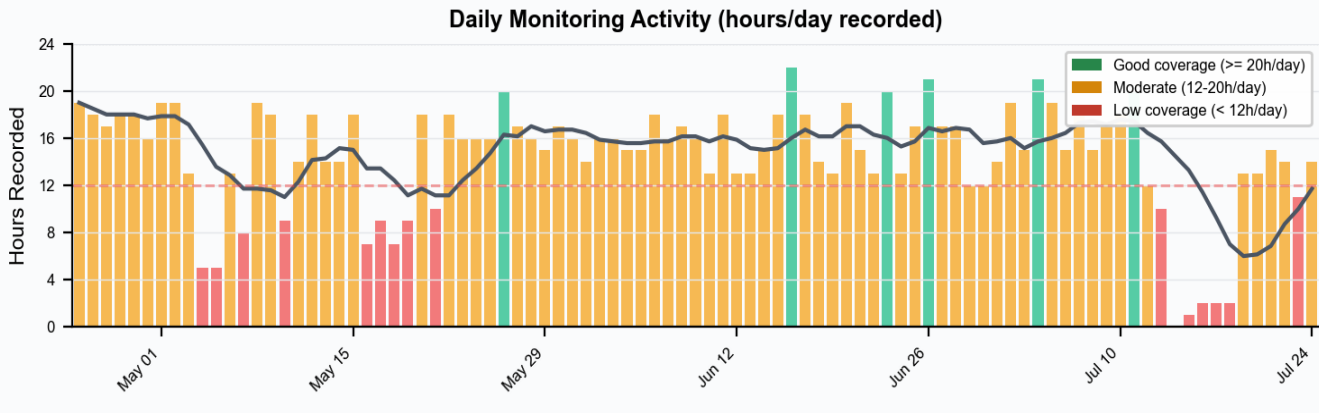


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.